IMP - Internal Monitoring Plan

Plan identification, purpose and boundary

IMP · v1.0

Study / protocol	CIP version	Site / code	IMP ID
Version	**Status** ☐ Draft ☐ Approved	**Supersedes (ID / version)**	**Effective from**
Next scheduled plan review	**Plan owner / role**	**Approved by / date**	**Controlled location**

A Purpose and use

The IMP is the site's plan for checking its own work during the investigation, alongside the sponsor's monitoring. It is written and approved before the first participant is included. It does not record any review: each review is run and recorded in the Internal Monitoring Checklist (IMC), which cites the IMP version it followed.

For each of the twelve checks, and any check the site adds, the plan fixes in advance what is checked (C), when, by whom, on which records and how many (D), and what counts as done, evidenced and effective (E). It also sets what calls a review early (F) and where each kind of result goes (G). Rules are set before results exist. A change applies only from its effective date onward and is entered in I; reviews already done stay with the version they used.

Unit of analysis	Decision supported	Inputs	Outputs
One site and one investigation, under one approved version of the plan.	Whether the site's own verification is defined well enough to catch, in time, a failure in a control the site is responsible for.	Site risk register (SRR), delegation log and training records, the CIP and its manuals, and the records each check reads.	The rules every IMC review follows. Findings go to the IMC, corrective actions to the CAPA log, plan changes back to I.

B Plan boundary

What the plan covers and what it rests on. The plan covers controls the site is responsible for; it does not replace sponsor monitoring.

Investigation(s), sites and phases covered	
Documents the plan is built on (ID / version)	
Sponsor monitoring: what the sponsor checks and how its findings reach the site	
Known constraints (staff, time, access to records)	
If a record picked for review is unavailable (rule for every check)	
Outside this plan	

First review · due after the first _____ inclusions or procedures, before routine reviews start, to catch workflow failures while few participants are exposed.

C Checks in scope

Keep every check unless it truly does not apply. A check left out needs the reason, who agreed and what would bring it back. List the SRR risks whose controls each check verifies; an SRR control with no check here is a gap to raise.

#	Check	Included?	Control to verify (what must have happened)	Source record(s) used	SRR risk IDs covered	If not included: reason, agreed by, condition to include
1	**Participant entry: consent and eligibility** Consent signed on the current version before any study procedure; eligibility confirmed by the PI before surgery	☐ Included ☐ Not included ☐ Deferred until:				
2	**Delegation and training** Everyone who did a study task was delegated and released for it on that date	☐ Included ☐ Not included ☐ Deferred until:				
3	**Sterile release to use** Integrity, expiry and storage confirmed before each device is used	☐ Included ☐ Not included ☐ Deferred until:				
4	**Implant traceability and component reconciliation** Implanted components match labels, lot/UDI, accountability and CRF	☐ Included ☐ Not included ☐ Deferred until:				
5	**Instrumentation and operating-room readiness** Instruments available, compatible and checked; substitutions recorded	☐ Included ☐ Not included ☐ Deferred until:				
6	**Operative documentation** Operative note lets a reader rebuild the final configuration and intraoperative decisions	☐ Included ☐ Not included ☐ Deferred until:				
7	**Imaging usability and timing** Image sets acquired in window, with the required views, and usable	☐ Included ☐ Not included ☐ Deferred until:				

C Checks in scope (continued)

#	Check	Included?	Control to verify (what must have happened)	Source record(s) used	SRR risk IDs covered	If not included: reason, agreed by, condition to include
8	**Follow-up execution** Visits done in window with all endpoint-critical components	☐ Included ☐ Not included ☐ Deferred until:				
9	**Endpoint data agreement** Endpoint-critical fields agree between source and CRF	☐ Included ☐ Not included ☐ Deferred until:				
10	**Timeliness and closure** Notes, data entry, queries, safety follow-up and actions closed on time	☐ Included ☐ Not included ☐ Deferred until:				
11	**External context** Safety notices, field actions and new literature on the device were looked for	☐ Included ☐ Not included ☐ Deferred until:				
12	**Complication monitoring** Complication reviews took place as planned and their signals were sent on	☐ Included ☐ Not included ☐ Deferred until:				
13		☐ Included ☐ Not included ☐ Deferred until:				
14		☐ Included ☐ Not included ☐ Deferred until:				

Rows 13 and 14 are for a check the site adds because a risk in the SRR is not covered by checks 1–12. Use the same row number in C, D, E and F.

D Schedule, reviewer and sample

Time each check by how fast its failure would spread. The reviewer is named by role, released for the task, and does not review work they did. Unit and volume: what is counted (procedure, image set, visit, field, event…) and how many are expected per period. Selection method: how the units reviewed are chosen. Sample size: the minimum reviewed each cycle, and when every unit is reviewed instead (a small population is reviewed in full). Expansion trigger: what makes the sample larger.

#	Check	Timing basis	Routine frequency	Early-review trigger	Reviewer / backup (roles)	Unit and volume	Selection method	Sample size	Expansion trigger
1	**Participant entry: consent and eligibility** Consent signed on the current version before any study procedure; eligibility confirmed by the PI before surgery	☐ Calendar ☐ Case count ☐ Each event ☐ Milestone							
2	**Delegation and training** Everyone who did a study task was delegated and released for it on that date	☐ Calendar ☐ Case count ☐ Each event ☐ Milestone							
3	**Sterile release to use** Integrity, expiry and storage confirmed before each device is used	☐ Calendar ☐ Case count ☐ Each event ☐ Milestone							
4	**Implant traceability and component reconciliation** Implanted components match labels, lot/UDI, accountability and CRF	☐ Calendar ☐ Case count ☐ Each event ☐ Milestone							
5	**Instrumentation and operating-room readiness** Instruments available, compatible and checked; substitutions recorded	☐ Calendar ☐ Case count ☐ Each event ☐ Milestone							
6	**Operative documentation** Operative note lets a reader rebuild the final configuration and intraoperative decisions	☐ Calendar ☐ Case count ☐ Each event ☐ Milestone							
7	**Imaging usability and timing** Image sets acquired in window, with the required views, and usable	☐ Calendar ☐ Case count ☐ Each event ☐ Milestone							

D Schedule, reviewer and sample (continued)

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E Criteria the reviewer applies

Write each criterion so two reviewers would reach the same answer. The IMC rates each check on these three questions separately.

#	Check	Execution: the control was done when…	Evidence: the record is sufficient when…	Effective: the control worked when…	Records the reviewer cites in the IMC
1	**Participant entry: consent and eligibility** Consent signed on the current version before any study procedure; eligibility confirmed by the PI before surgery				
2	**Delegation and training** Everyone who did a study task was delegated and released for it on that date				
3	**Sterile release to use** Integrity, expiry and storage confirmed before each device is used				
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E Criteria the reviewer applies (continued)

#	Check	Execution: the control was done when…	Evidence: the record is sufficient when…	Effective: the control worked when…	Records the reviewer cites in the IMC
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13					
14					

Rows 13 and 14 are for a check the site adds because a risk in the SRR is not covered by checks 1–12. Use the same row number in C, D, E and F.

F Triggers for an earlier review

Any trigger marked Yes calls a review before the next scheduled one, for the checks listed. A minor observation is not a trigger.

Trigger	Applies?	Checks reviewed early (#)	Called by (role)	Within (days)
Serious adverse event or serious device effect at the site	☐ Yes ☐ No			
Complication cluster or signal from the CSST	☐ Yes ☐ No			
Important protocol deviation	☐ Yes ☐ No			
Repeated endpoint data discrepancy	☐ Yes ☐ No			
Pattern of unusable or late images (IQL)	☐ Yes ☐ No			
Device, instrument or equipment malfunction	☐ Yes ☐ No			
CIP amendment	☐ Yes ☐ No			
Departure or change of a key person or of a reviewer	☐ Yes ☐ No			
Sponsor finding that needs a local response	☐ Yes ☐ No			
Corrective action that failed or was reopened	☐ Yes ☐ No			
	☐ Yes ☐ No			

G Where each result goes

For each result the IMC can give, write the minimum response the site commits to, the record it goes to, and who owns it.

Result in the IMC	Minimum response and record it goes to	Owner (role)
Complete, adequate and effective		
Observation: isolated, low consequence, corrected at review		
Partial execution or partial evidence		
Not performed, or evidence missing		
Not effective		
Not assessable		
Recurrent, across units, or with material consequence		
Possible harm to a participant or a reportable event		

IMP - Internal Monitoring Plan

Readiness, approval and changes

H Ready to start

Confirm before the first inclusion. PI approval means the plan can be run with the people and time available.

Confirmation	Status	Date, or action and due date if No
Every reviewer and backup is released for the task in the delegation log	☐ Yes ☐ No	
Staff who do the checked work know the plan	☐ Yes ☐ No	
The IMC is prepared with this IMP ID and version	☐ Yes ☐ No	
First review due (date or after inclusion number)		

Plan owner

Name / role:

Signature / date:

Principal Investigator

Name:

Signature / date:

I Changes to the plan

A change applies from its effective date. A result already seen is never the reason to loosen a rule for the same period.

Change ID	Section / check	Previous rule	New rule	Reason (IMC, CAPA, SRR, sponsor…)	Effective from	Approved by